



Transportation of mental health consumers

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Functional subgroup	Mental Health
Summary	Transporting mental health patients must be undertaken in a manner that is least restrictive to the patient and maintains the safety of both patients and escorting staff in accordance with the Mental Health Act 2014 and Department of Health policy.
Policy owner	Executive Director Clinical Service Planning and Population Health
Contact	Anureet Johl, 6152 1169
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NSQHS Standards	Clinical Governance Comprehensive Care Communicating for safety Recognising and responding to acute deterioration
NSMHS Standards	Standard 2: Safety Standard 3: Consumer and carer participation
Chief Psychiatrist's Standards for Clinical Care	Risk assessment and management
Status	Approved

1. Background

1.1. About this document

Transporting mental health patients must be undertaken in a manner that is trauma informed, least restrictive to the patient and maintains the safety of both patients and escorting staff in accordance with the Mental Health Act 2014 and Department of Health policy.

2. Policy Statement

This document outlines the principles and processes for providing/arranging transport (where clinically appropriate) for mental health patients in government vehicles to facilitate their assessment, examination, or treatment.

The decision to transport a patient in a government vehicle is a clinical decision and must be fully documented in the patient's digital medical record (DMR).

2.1. Reasons for transporting patients

May include:

- transfer to another health care facility, including a private hospital or a nursing home.
- to support attendance at outpatient appointments to minimise the risks of non-compliance with treatment and / or medication regimes or to maintain and treat the physical health care needs of the patient
- support participation in group treatment activities
- as a component of a person-centered care including individualised care coordination activities
- transporting the patient to the Mental Health Service for assessment and / or admission (as either a voluntary or an involuntary patient)

Refer to:

- [SMHS Care Coordination in Mental Health Policy.](#)
- [Mental Health Act 2014](#)
- [Clinicians' Practice Guide to the Mental Health Act 2014](#)

2.2. Risk assessment

A Risk Assessment and Management Plan (RAMP) must be completed in webPSOLIS to identify potential transport risks prior to the patient being transported in a government vehicle by health service staff, or as soon as possible.

The staff member transporting/escorting the patient is responsible for ensuring that the management plan developed to mitigate identified risks is documented on the RAMP and in the patient's DMR.

Where the patient has been deemed at clear and imminent risk of unpredictable behaviour including absconding or threat of violence during the escort, they must not be transported by SMHS staff and require the involvement of other services (refer to Section 4).

Clinicians are responsible for complying with the procedures and protocols adopted by the Mental Health Service to ensure the safety of patients and staff when transporting a patient/carer in a government vehicle.

- elements that must be considered in the risk assessment and management plan prior to transport in a government vehicle include: the patient's mental state, mood and level of agitation.
- any history of impulsive or unpredictable behaviour including absconding and/or aggression
- any current history of alcohol or drug use.
- lack of history relating to the patient.
- principles of trauma-informed care are to be balanced with any unaddressed risks when considering alternative options for transportation.
- consideration of physical needs (i.e., disability) impact/ extra consideration for type of vehicle used (wheelchair access).

Where a RAMP already exists in webPSOLIS it must be reviewed to ensure the assessment and plan are current.

2.3. Inclusion of carers, close family members or other personal support persons

When using a government vehicle and where clinically appropriate it may be beneficial to have a carer, family member or other personal support person accompany the patient e.g., from home to the health service (for assessment or admission). In this instance that person may be invited to accompany the patient and clinician.

Where the patient is responsible for the care of a minor or other dependent, and having the minor accompany them to the health service is in the best interests of the dependent minor, the clinician may consider taxi transport as they are licensed for appropriate safety equipment e.g., baby capsule, child seat etc.

2.4. Gender and cultural considerations

The gender and cultural needs of the patient and of the staff involved in the transportation must be given careful consideration. This is especially important in relation to patients who are minors.

Where gender issues cannot be addressed using available staff, advice should be sought from senior staff on risk mitigation strategies.

2.5. Escorts

The patients assessed level of risk will determine:

- the number of staff required for the transport
- the level of staff skill and knowledge required for the transport
- mode of transport

Clinicians must:

- be informed of the patient's clinical condition and receive appropriate clinical handover
- be fully aware of potential risks posed by a patient.
- implement identified risk management strategies including access to duress
- report any risk escalation and seek appropriate support

Where a patient absconds while being transported, staff should refer to the SMHS Missing or Suspected Missing Mental Health Patients Policy.

Where risk may be mitigated by an additional escort when transporting the patient in a government vehicle, the patient is to be seated in the rear passenger-side seat with the escort in the rear driver's-side seat. If there are two escorts, they should sit either side of the patient in the rear seat.

Where clinically appropriate it may be beneficial to have a personal support person accompany the patient, refer to subheading 2.3 for further details.

3. Roles and Responsibilities

3.1. Executive

Each SMHS site or service executive director will ensure that managers and supervisors are aware of the requirement to implement guidelines and safe work procedures relating to the Transportation of mental health consumers.

3.2. Managers/supervisors

Managers and supervisors should:

- Undertake risk assessments of tasks involving the Transportation of mental health consumers
- Ensure the development of relevant protocols and procedures
- Ensure staff are trained in relevant safe work procedures and manual tasks

3.3. Staff

Staff should:

- Fully document the decision to transport a patient in a government vehicle

- Comply with mandatory training requirements and proactively seek further training as required
- Comply with safe work procedures
- Ask for assistance if required and involve relevant medical staff in the decision-making process regarding patient transportation

4. Transport Options

Where the risk assessment indicates that it is unsafe to transport the patient in a government vehicle, even with the presence of an escort(s) to support the driver, the patient should be transported by the Inter Hospital Patient Transport (IHPT), ambulance or police services.

IHPT or Police assistance with transportation of mental health patients under the MH Act 2014 must comply with the requirements of [MP 0063/17 Requesting Transport for Mental Health Patients Subject to Transport Orders Policy](#)

Where the involvement of another contracted transport provider, transport officer or police is deemed necessary a [Mental Health Patient Transport Risk Rating Form](#) must be completed. Refer to the [Department of Health Mental Health patient Transport Hub page](#) for all transport related documentation.

In situations when risk is deemed significant discussion should occur with medical staff to confirm risk category and subsequently if transport Officers and or WA Police involvement will be required as set out in the MHA 2014 section 149 (2).

4.1. Transport Officers

The Mental Health Regulations 2015 may authorise a person, or a person in a class of person (a “transport officer”), to carry out a transport order.

An individual in the community or in a mental health inpatient service who is subject to a Transport Order and who has been assessed by a psychiatrist, Medical Practitioner or authorised Mental Health Practitioner (AMHP) as Low, Medium or High risk must be provided with an Inter Hospital Patient Transport (IHPT) Service in the company of a Transport Officer (WA Police to be contacted if patients deemed as significant risk).

A Transport Officer can only authorise a Police Officer instead of a Transport Officer to carry out the order if the practitioner (Medical or AMHP) or psychiatrist making the order is satisfied:

- There is significant risk of serious harm to the person being transported or to another person **or**
- a Transport Officer will not be available to carry out the order within a reasonable time **and** any delay in carrying out the order beyond that time is likely to pose a significant risk of harm to the person being transported or to another person.

Refer to [Mental Health Act 2014 Information for Transport Officers](#)

Where a consumer/ client must be transported to hospital, the health service is responsible for arranging the mode of transport.

The providers of these services are:

Non-Emergency Inter hospital transport:

- Wilson Medic One
- National Patient transport

Mental Health Transport Services (patients under MHA, 2014):

- Wilson Medic One

4.2. Ambulance

Where a consumer/client must be transported in an ambulance to hospital for physical health concerns, the health service is responsible for arranging the ambulance.

The providers of these services are:

Emergency (any referrer):

- St John Ambulance

Where an ambulance is not available in a timely manner for transportation under the Mental Health Act 2014, this should be escalated directly to the Duty Manager Ambulance Operations.

Refer to [Mental Health Act Regulations 2015](#).

Clinical Handover:

A detailed clinical handover between the attending mental health professional and transport / ambulance officers should include:

- briefing on the patient's physical and mental state
- relevant personal details and next of kin contact details
- details regarding any sedation administered and potential need for restraint
- risk assessment
- transport and inpatient admission requirements
- the patient's legal status
- the nature of any documentation that will accompany the patient
- the name of any receiving clinician or service expecting the patient.

Communication in these circumstances is usually face-to-face discussion using the ISoBAR format between the various professionals at the scene as per [Clinical Handover Policy MP0095](#).

4.3. Police

Transport of a person with mental health concerns by police should always be the option of last resort when all other transport options are considered unsuitable.

A psychiatrist, medical practitioner or AMHP who assesses an individual as 'Significant' risk must request WA Police assistance. Priority for assistance will be determined according to WA Police operational demands.

Police may request that the patient be transported in a government vehicle with the police as an escort. Consumers will only be transported in police vehicles if the attending police:

- determine there are no other vehicles available.
- have to restrain the consumer; and
- have identified a problem on arrival or during the interview that confirms the need to be conveyed in the police vehicle.

For patients on a Form 7D Apprehension and Return Order police should be directed to:

- contact the service that completed the Form 7D for further information.
- return the patient to ward or other place specified on the Form 7D
- implement the backup arrangement, which is to take the consumer to the Emergency Department of a hospital, preferably the one in their catchment area.

Apprehension and transportation of the referred person by the police must be completed within 72 hours of the person being referred to an authorised hospital.

The assessing practitioner should contact the Community Automatic Dispatch (CAD) Coordinator on 131 444 to request Police Transport which will result in a CAD job on the Police system.

The MH Act 2014 Form 4A Transport Order and MH Patient Transport Risk Rating Form are then emailed to Communications Duty Inspector via:

communications.Mental.Health.Smail@police.wa.gov.au who forwards the forms to the Radio Supervisor (who updates the CAD job to confirm forms are received by police). When a vehicle becomes available, they are dispatched to the job.

A copy of the Form 4A will be provided to:

- The person as soon as practicable and clinically appropriate
- the Transport Officer or police responsible for carrying out the transport
- the Personal Support Person will be notified as soon as practicable and clinically appropriate

Refer to [Mental Health Act 2014 section 150\(2\)](#).

5. Significant risk and police not available

WA Police will prioritise their response time based on police service priorities. Therefore, requesting health services must manage identified risks while awaiting attendance by WA police.

In situations when the risk is deemed significant and police report that they are unavailable, clinicians should discuss the patient's management plan with senior medical staff to inform them of the risk. The medical staff should contact the Community Automatic Dispatch (CAD) Supervisor on 131 444 to inform them of the risk.

The Duty Control Commander at WA Police may be contacted only if a situation occurs where the mental health service considers reasonable assistance is not being provided.

6. Use of Safety Equipment

Staff transporting mental health patients and anyone accompanying them must ensure that everyone in the vehicle is safely restrained during the journey e.g., by seat belts, child restraints etc.

Refer to Road Traffic Code 2000, Part 16, Division 2 and sections 133-234.

7. Documentation

The responsible mental health clinician must ensure that, all relevant documentation is completed.

Clinicians are to fully document the clinical rationale underpinning any decision to transport mental health patients in the patient's DMR.

The clinician will provide the transport officer with a copy of the transport order and the Mental Health Transport Risk Rating Form.

The receiving service will ensure the transport order and Mental Health Transport Risk Rating Form are filed in the patient's DMR.

Escort arrangements are to be documented in the DMR.

8. Transport order

Where indicated, under section 148(2) of the Mental Health Act 2014, the practitioner or psychiatrist who makes a transport order in respect of a person must give a copy of the order to:

- the person; and
- the transport officer or policy officer responsible for carrying out the order.

9. Reporting of incidents or accidents

All clinical incidents or near misses occurring during patient transfer must be reported on the Datix Clinical Incident Management System.

Any incidents or accidents impacting staff that occur during transportation are to be managed and reported in accordance with health service provider's protocols and procedures which will include the completion of CHoIR documentation if appropriate.

Refer to:

- Department of Health Clinical Incident Management policy 2019
- SMHS Missing or Suspected Missing Mental Health Patient Policy.
- SMHS WHS Hazard and Incident Management procedure

10. Compliance monitoring and legislative penalties

10.1. Compliance Monitoring

The Executive of each SMHS mental health service will be responsible for monitoring compliance.

Compliance monitoring methods may include the following:

- Incident or near miss events during patient transportation processes.
- Incidences of patient absconding (whilst under transport conditions)
- Incidents of serious assault by staff or patient (during transport).

Note: these events would be reported as per relevant incident reporting processes:

- Reporting to the Office of the Chief Psychiatrist (OCP) as per requirements.

Refer to: [OCP Policy for Mandatory Reporting 2018](#)

10.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

11. Additional references

11.1. Legislation

- Mental Health Act 2014 sections 29, 63, 67, 92, 112, 129, 133, 148, 150.
- Road Traffic Code 2000, Part 16, Division 2 and sections 133-234

11.2. Department of Health

- [Use of Physical and/or Mechanical Restraint during Road-based Transportation of Mental Health Patients Policy](#)
- [MP 0063/17 Requesting Road-Based Transport for Mental Health Patients Subject to Transport Orders Policy](#)
- [DOH Clinical Incident Management 2019](#)

11.3. SMHS

- SMHS Missing or Suspected Missing Mental Health Patient Policy.
- SMHS Care Coordination in Mental Health Policy.
- SMHS Rights of Patients in Mental Health Policy

11.4. Other

- [EMHS Mental Health Transport](#)
- [Transport officer/transport booking process](#)

12. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
MP V4	13/09/2023	08/09/2023	08/09/2026	08/09/2026	Original version

13. Approval

Approval by	SMHS AEG – Kate Gatti- Executive Director Clinical Services
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This document can be made available in alternative formats on request.

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